

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Aciclovir cream and tablets for the treatment of Cold Sores (Herpes Labialis)

Summary of NICE CKS Guidelines for Cold Sores

Overview

- Cold sores are caused by **Herpes Simplex Virus type 1 (HSV-1)**.
- Common, self-limiting **viral infection** of the lips and perioral area.
- May be **recurrent** after primary infection due to virus reactivation.

Symptoms

- **Prodromal symptoms:** tingling, burning, or itching
- **Painful vesicles** on lips or around the mouth
- Vesicles rupture, crust over, and heal within **7–10 days**
- May be associated with **fever, malaise, swollen lymph nodes** in primary infection

Triggers for Recurrence

- Sun exposure
- Stress
- Illness (e.g. viral infections)
- Menstruation
- Immunosuppression

Diagnosis

- **Clinical diagnosis** based on typical presentation.
- Laboratory testing (PCR or viral culture) only if:
 - Atypical presentation
 - Immunocompromised patient

- o Severe or persistent lesions
-

Management

Mild/Typical Cold Sores

- Usually **no treatment required** — self-limiting.
- **Topical antiviral cream** (e.g. **aciclovir 5%** or **penciclovir 1%**):
 - o Apply **as early as possible** (within 48 hours of symptom onset)
 - o Shortens duration by 1–2 days

Frequent or Severe Episodes

- Consider **oral antivirals** (e.g. **aciclovir 200 mg 5 times/day for 5 days** or **valaciclovir 500 mg BD for 5 days**)
- For **frequent recurrences** (≥ 6 /year), consider **suppressive therapy**:
 - o e.g. aciclovir 400 mg BD

Supportive Measures

- Keep the area clean and dry
 - Use lip balm or moisturiser
 - Avoid kissing or close contact while lesions are present
-

When to Refer or Seek Specialist Advice

- Immunocompromised patients (risk of severe or systemic HSV)
 - Cold sores lasting >10 days or spreading
 - Severe pain or systemic symptoms
 - Ophthalmic involvement (refer urgently to ophthalmology)
 - Atypical, persistent, or extensive lesions
-

Prevention Advice

- Use sunblock lip balm if sunlight is a trigger
- Avoid sharing utensils, towels, or kissing when lesions are present
- Avoid triggers where possible

Patient Group Direction

for the supply/administration of

Aciclovir cream

for the treatment to

Cold Sores (Herpes Labialis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Aciclovir cream is indicated for the treatment of recurrent herpes labialis (cold sores) of the lips and face in adults and adolescents.
Inclusion criteria	- Adults and adolescents aged 12 years and over. - Clinical diagnosis of recurrent herpes labialis. - Informed consent obtained. - No contraindications to aciclovir.
Exclusion criteria	- Known hypersensitivity to aciclovir, valaciclovir, or any excipients. - Immunocompromised patients or severe recurrent episodes. - Use on mucous membranes (e.g. eyes, inside mouth, genitals). - Pregnancy or breastfeeding unless assessed as appropriate by the prescriber.
Cautions	- Start treatment at the first sign of symptoms (e.g. tingling or itching). - Avoid contact with eyes or mucous membranes. - Do not share tubes to prevent cross-infection. - Counsel on hygiene measures to prevent spread of infection.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Aciclovir cream 5%
Legal category	POM
Storage	Store below 25°C. Do not refrigerate or freeze.
Route/method of administration	Topical application to affected area of the lips and face. Patients should wash their hands before and after applying the cream and avoid unnecessary rubbing of the lesions or touching with a towel, to avoid aggravating or transferring the infection.
Dose and frequency	Apply to affected area five times daily at approximately four-hour intervals for 5 days.
Quantity to be administered and/or supplied	Up to 1 x 2 g or 5 g tube per episode.
Maximum or minimum treatment period	Use for up to 5 days. If lesions have not healed, treatment may be continued for up to 10 days. If lesions are still present after 10 days, users should be advised to consult a doctor.
Adverse effects	Mild stinging or burning, drying or flaking of skin. Rare: erythema, allergic contact dermatitis. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Advise the person/carers to seek medical advice if their symptoms worsen (for example the lesion spreads, new lesions develop, or there is persistent fever or difficulty taking fluids), or no significant improvement is seen after 5–7 days. Reassure the person/carers that oral herpes simplex infections are usually self-limiting, and that lesions should heal without scarring. Advise on measures for symptom relief, such as: - Paracetamol and/or ibuprofen to relieve pain and fever, if required, and there are no contraindications. - Adequate fluid intake to reduce the risk of dehydration. Give advice to minimize the risk of transmission to other people and autoinoculation: - Explain that herpes simplex virus is easily transmitted to other people. - Avoid kissing and oral sex until all lesions have fully healed. - Do not share items that come into contact with lesions (for example makeup and lip balms). - Avoid touching the lesions, other than when applying topical preparations, which should be dabbed on rather than rubbed in to minimize mechanical trauma to the lesions. - Topical treatments should not be shared with other people. - Wash hands with soap and water immediately after touching lesions. - Take care if using contact lenses, as there is a risk of transmission to the eye if lenses become contaminated.

	- Defer elective dental treatment until all lesions have fully healed. - Advise the person to try to avoid trigger factors, if possible. If sunlight is a trigger, advise the use of sunscreen or sunblock lip balm (sun protection factor 15 or greater), to help reduce recurrent episodes.
--	--

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
1/11/25		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025

Patient Group Direction

for the supply/administration of

Aciclovir 200mg

for the treatment to

Cold Sores (Herpes Labialis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Aciclovir tablets are indicated for the treatment of recurrent herpes labialis (cold sores) of the lips and face in adults and adolescents.
Inclusion criteria	- Adults and adolescents aged 12 years and over. - Clinical diagnosis of recurrent herpes labialis. - Informed consent obtained. - No contraindications to aciclovir.
Exclusion criteria	- Known hypersensitivity to aciclovir, valaciclovir, or any excipients. - Immunocompromised patients or severe recurrent episodes. - Use on mucous membranes (e.g. eyes, inside mouth, genitals). - Pregnancy or breastfeeding unless assessed as appropriate by the prescriber.
Cautions	- Start treatment at the first sign of symptoms (e.g. tingling or itching). - Counsel on hygiene measures to prevent spread of infection. - Caution is advised when administering aciclovir to patients with impaired renal function and adequate hydration should be maintained. - Elderly patients are likely to have reduced renal function and therefore caution must be considered in this group of patients too. - Tablets may be dispersed in a minimum of 50 ml of water or swallowed whole with a little water.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Aciclovir 200mg tablets
Legal category	POM
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration with or without food.
Dose and frequency	200 mg five times daily for 5 days.
Quantity to be administered and/or supplied	25 tablets per episode.
Maximum or minimum treatment period	Typically 5 days; may be extended to 10 days in severe or immunocompromised cases.
Adverse effects	Nausea, vomiting, diarrhoea, headache, dizziness. Rare: rash, fatigue, renal dysfunction. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Advise the person/carers to seek medical advice if their symptoms worsen (for example the lesion spreads, new lesions develop, or there is persistent fever or difficulty taking fluids), or no significant improvement is seen after 5–7 days. Reassure the person/carers that oral herpes simplex infections are usually self-limiting, and that lesions should heal without scarring. Advise on measures for symptom relief, such as: - Paracetamol and/or ibuprofen to relieve pain and fever, if required, and there are no contraindications. - Adequate fluid intake to reduce the risk of dehydration. Give advice to minimize the risk of transmission to other people and autoinoculation: - Explain that herpes simplex virus is easily transmitted to other people. - Avoid kissing and oral sex until all lesions have fully healed. - Do not share items that come into contact with lesions (for example makeup and lip balms). - Avoid touching the lesions, other than when applying topical preparations, which should be dabbed on rather than rubbed in to minimize mechanical trauma to the lesions. - Topical treatments should not be shared with other people. - Wash hands with soap and water immediately after touching lesions. - Take care if using contact lenses, as there is a risk of transmission to the eye if

	lenses become contaminated. - Defer elective dental treatment until all lesions have fully healed. - Advise the person to try to avoid trigger factors, if possible. If sunlight is a trigger, advise the use of sunscreen or sunblock lip balm (sun protection factor 15 or greater), to help reduce recurrent episodes.
--	---

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
1/11/25		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025